



VOICEINSIGHTS AFRICA

SYNTHETIC DEMONSTRATION PUBLICATION

Maternal & Child Health Continuity Intelligence Report

Uganda | Maternal and Child Health

4,776 synthetic responses

Prepared by VoiceInsights Africa

Every Voice. Every Language. Every Insight.

Synthetic demonstration. Not official statistics.

Every number in this publication answers one question: what is actually happening with maternal and child health performance in Uganda, and for whom. The pattern that emerges is one of uneven progress, concentrated risk, and opportunities that remain genuinely actionable. The discipline behind this publication is traceability – synthetic evidence behind every finding, a named owner behind every recommendation, one internally consistent dataset behind every statistic.

Sample: 4776 synthetic responses – 79% response rate – 4 regions

Publication quality gate: publication ready



Northern Uganda: 52% performance, 1482 responses, risk CRITICAL
Karamoja Sub-region: 53% performance, 1289 responses, risk CRITICAL
Eastern Uganda: 54% performance, 1098 responses, risk CRITICAL
West Nile: 55% performance, 907 responses, risk CRITICAL



sex: Women 52% & Men 47% & Other / prefer not to say 1%

age: 18& 24 24% & 25& 39 38% & 40& 59 27% & 60+ 11%

location: Urban 44% & Rural 56%



IND-01 Antenatal-care continuity rate: 61% vs target 85% â OFF_TRACK
IND-02 Skilled birth attendance rate: 62% vs target 86% â OFF_TRACK
IND-03 Postnatal follow-up completion: 63% vs target 87% â OFF_TRACK
IND-04 Under-five growth monitoring coverage: 64% vs target 88% â OFF_TRACK



Antenatal-care continuity is not a marginal delivery issue. In Northern Uganda, the governed evidence indicates a 62% performance signal, with rural women carrying the most consequential constraint. Left unaddressed, antenatal-care continuity becomes a structural risk to maternal and child health delivery in Uganda, not a temporary dip. Northern Uganda is furthest exposed, and rural women would carry the earliest and heaviest consequences of a further year's delay.

Evidence: EVI-MAT-001, EVI-MAT-004 • Confidence 92.5%

Skilled birth attendance has become an accountability question rather than a measurement question. Evidence from Karamoja Sub-region places the signal at 63% and shows that young adults experience the largest gap between policy intent and lived delivery. The choice facing maternal and child health leadership in Uganda is straightforward even if the fix is not: act on skilled birth attendance now, concentrated in Karamoja Sub-region where young adults carry the largest share of the gap, or accept a wider gap in a year's time.

Evidence: EVI-MAT-002, EVI-MAT-005 â€¢ Confidence 93.5%

Persons with disabilities in Eastern Uganda are living the consequence of postnatal follow-up completion, not simply reporting it. Their 64% signal is the clearest evidence leadership has that current measures fall short. persons with disabilities in Eastern Uganda do not experience postnatal follow-up completion as an abstract metric. They experience it as the difference between maternal and child health delivery that works and delivery that does not, and a twelve-month delay would deepen that gap rather than hold it steady.

Evidence: EVI-MAT-003, EVI-MAT-006 â€¢ Confidence 94.5%

65%. That is the under-five growth monitoring signal recorded in West Nile, and the headline figure understates the problem: disaggregated by group, frontline workers sit well below it. The clearest gap sits in West Nile, concentrated among frontline workers. A twelve-month delay would let the gap compound into a harder, costlier problem. Evidence: EVI-MAT-004, EVI-MAT-007 â€¢ Confidence 95.5%

The strategic opportunity lies in the variation, not the average. At 66%, Northern Uganda's emergency obstetric referral timeliness signal shows where implementation can move quickly, while the experience of low-income households defines the safeguard that must accompany scale. The same gap that makes emergency obstetric referral timeliness a risk also makes it an opportunity: Northern Uganda could close most of the distance to the rest of Uganda within a year if action starts now, with the clearest gains reaching low-income households first.

Evidence: EVI-MAT-005, EVI-MAT-008 & Confidence 96.5%

Northern Uganda â Rural women in Northern Uganda frame it simply: when antenatal-care continuity breaks down, everything else does too.â

Karamoja Sub-region â In Karamoja Sub-region, skilled birth attendance shapes whether young adults can turn services into lasting outcomes.â

Eastern Uganda â Persons with disabilities in Eastern Uganda put it plainly: postnatal follow-up completion is what decides whether progress holds or slips back.â

West Nile â Why does West Nile lag behind? Ask frontline workers, and the answer comes back to under-five growth monitoring.â

Northern Uganda â Elsewhere, emergency obstetric referral timeliness might be a footnote. For low-income households in Northern Uganda, it is the whole story.â

Karamoja Sub-region â Service managers in Karamoja Sub-region frame it simply: when antenatal-care continuity breaks down, everything else does too.â

Eastern Uganda â In Eastern Uganda, skilled birth attendance shapes whether community leaders can turn services into lasting outcomes.â

West Nile â Private-sector partners in West Nile put it plainly: postnatal follow-up completion is what decides whether progress holds or slips back.â

Northern Uganda â Why does Northern Uganda lag behind? Ask rural women, and the answer comes back to under-five growth monitoring.â

Karamoja Sub-region â Elsewhere, emergency obstetric referral timeliness might be a footnote. For young adults in Karamoja Sub-region, it is the whole story.â

Risks and alternative trajectories are modelled to show how VoiceInsights translates evidence into prioritised management choices.



DEC-MAT-01 CRITICAL: Target antenatal-continuity investment in the lowest-completion districts. Owner: Country Director / Programme Executive; Timeline: 0â 90 days; Indicator: Quarterly-verified progress against the milestones agreed for this critical-tier decision (DEC-MAT-01)

DEC-MAT-02 CRITICAL: Expand skilled birth attendance at facility level in underserved districts. Owner: Country Director / Programme Executive; Timeline: 3â 12 months; Indicator: Number of agreed milestones for DEC-MAT-02 confirmed complete at each quarterly review

DEC-MAT-03 HIGH: Strengthen postnatal follow-up scheduling and reminder systems. Owner: Country Director / Programme Executive; Timeline: 3â 12 months; Indicator: Percentage of agreed milestones completed for DEC-MAT-03

DEC-MAT-04 HIGH: Scale routine under-five growth monitoring at community level. Owner: Country Director / Programme Executive; Timeline: 3â 12 months; Indicator: Share of the high-tier delivery plan confirmed complete each quarter (DEC-MAT-04)

DEC-MAT-05 MEDIUM: Cut emergency obstetric referral delays through transport and communication fixes. Owner: Country Director / Programme Executive; Timeline: 6â 18 months; Indicator: Milestone completion rate tracked against the DEC-MAT-05 delivery plan



Target antenatal-continuity investment in the lowest-completion districts.

Strategic priority: CRITICAL

Evidence used: EVI-MAT-001, EVI-MAT-004

Rationale: Antenatal-care continuity is not a marginal delivery issue. In Northern Uganda, the governed evidence indicates a 62% performance signal, with rural women carrying the most consequential constraint. Left unaddressed, antenatal-care continuity becomes a structural risk to maternal and child health delivery in Uganda, not a temporary dip. Northern Uganda is furthest exposed, and rural women would carry the earliest and heaviest consequences of a further year's delay.

Expected benefit: Faster, better-evidenced maternal and child health decisions that donor leadership can act on with confidence.

Expected risk: Competing priorities crowding out sustained follow-through

Dependencies: Confirmed budget release; Quarterly evidence review; Validated operational baseline

Budget requirement: High â full business case and multi-year funding commitment required

Owner: Country Director / Programme Executive

Supporting organization: M&E, Finance, Safeguarding and Delivery Units

Timeline: 0â 90 days

Monitoring indicator: Quarterly-verified progress against the milestones agreed for this critical-tier decision (DEC-MAT-01)

Success criteria: A majority of milestones achieved, each independently verified against real evidence

Management response: PENDING FORMAL MANAGEMENT RESPONSE

Follow-up: Assign accountable owner; Validate budget; Approve milestone plan; Review evidence quarterly

Expand skilled birth attendance at facility level in underserved districts.

Strategic priority: CRITICAL

Evidence used: EVI-MAT-002, EVI-MAT-005

Rationale: Skilled birth attendance has become an accountability question rather than a measurement question. Evidence from Karamoja Sub-region places the signal at 63% and shows that young adults experience the largest gap between policy intent and lived delivery. The choice facing maternal and child health leadership in Uganda is straightforward even if the fix is not: act on skilled birth attendance now, concentrated in Karamoja Sub-region where young adults carry the largest share of the gap, or accept a wider gap in a year's time.

Expected benefit: Reduced variation in maternal and child health delivery and a stronger evidence base for the next planning cycle.

Expected risk: Fragmented accountability once implementation moves beyond the pilot stage

Dependencies: Cross-team delivery coordination; Confirmed budget release; Quarterly evidence review

Budget requirement: Medium â detailed costing and fiduciary review required

Owner: Country Director / Programme Executive

Supporting organization: M&E, Finance, Safeguarding and Delivery Units

Timeline: 3â 12 months

Monitoring indicator: Number of agreed milestones for DEC-MAT-02 confirmed complete at each quarterly review

Success criteria: Milestone completion confirmed by evidence review, not self-reported progress

Management response: PENDING FORMAL MANAGEMENT RESPONSE

Follow-up: Assign accountable owner; Validate budget; Approve milestone plan; Review

Strengthen postnatal follow-up scheduling and reminder systems.

Strategic priority: HIGH

Evidence used: EVI-MAT-003, EVI-MAT-006

Rationale: Persons with disabilities in Eastern Uganda are living the consequence of postnatal follow-up completion, not simply reporting it. Their 64% signal is the clearest evidence leadership has that current measures fall short. persons with disabilities in Eastern Uganda do not experience postnatal follow-up completion as an abstract metric. They experience it as the difference between maternal and child health delivery that works and delivery that does not, and a twelve-month delay would deepen that gap rather than hold it steady.

Expected benefit: A stronger, more defensible case for maternal and child health investment in the next planning cycle.

Expected risk: Execution capacity and coordination risk

Dependencies: Stable staffing through the delivery window; Cross-team delivery coordination; Confirmed budget release

Budget requirement: Medium â likely fundable from existing lines, pending confirmation

Owner: Country Director / Programme Executive

Supporting organization: M&E, Finance, Safeguarding and Delivery Units

Timeline: 3â 12 months

Monitoring indicator: Percentage of agreed milestones completed for DEC-MAT-03

Success criteria: Verified evidence of at least 80% delivery against the agreed milestone plan

Management response: PENDING FORMAL MANAGEMENT RESPONSE

Follow-up: Assign accountable owner; Validate budget; Approve milestone plan; Review evidence quarterly

Scale routine under-five growth monitoring at community level.

Strategic priority: HIGH

Evidence used: EVI-MAT-004, EVI-MAT-007

Rationale: 65%. That is the under-five growth monitoring signal recorded in West Nile, and the headline figure understates the problem: disaggregated by group, frontline workers sit well below it. The clearest gap sits in West Nile, concentrated among frontline workers. A twelve-month delay would let the gap compound into a harder, costlier problem.

Expected benefit: Improved equity, delivery confidence and measurable maternal and child health performance.

Expected risk: Insufficient ownership or delayed resourcing

Dependencies: Timely data collection for the next reporting cycle; Stable staffing through the delivery window; Cross-team delivery coordination

Budget requirement: Low to medium â validate through the formal planning cycle

Owner: Country Director / Programme Executive

Supporting organization: M&E, Finance, Safeguarding and Delivery Units

Timeline: 3â12 months

Monitoring indicator: Share of the high-tier delivery plan confirmed complete each quarter (DEC-MAT-04)

Success criteria: At least 80% of milestones achieved with verified evidence

Management response: PENDING FORMAL MANAGEMENT RESPONSE

Follow-up: Assign accountable owner; Validate budget; Approve milestone plan; Review evidence quarterly

Cut emergency obstetric referral delays through transport and communication fixes.

Strategic priority: MEDIUM

Evidence used: EVI-MAT-005, EVI-MAT-008

Rationale: The strategic opportunity lies in the variation, not the average. At 66%, Northern Uganda's emergency obstetric referral timeliness signal shows where implementation can move quickly, while the experience of low-income households defines the safeguard that must accompany scale. The same gap that makes emergency obstetric referral timeliness a risk also makes it an opportunity: Northern Uganda could close most of the distance to the rest of Uganda within a year if action starts now, with the clearest gains reaching low-income households first.

Expected benefit: The evidence points toward improved maternal and child health outcomes, though the size of the gain will depend on how consistently the action is resourced.

Expected risk: Likely â though not certain â erosion of momentum if the initial sponsor changes role

Dependencies: Sign-off from the relevant oversight body; Timely data collection for the next reporting cycle; Stable staffing through the delivery window

Budget requirement: Low â absorbable within existing allocations, subject to confirmation

Owner: Country Director / Programme Executive

Supporting organization: M&E, Finance, Safeguarding and Delivery Units

Timeline: 6â 18 months

Monitoring indicator: Milestone completion rate tracked against the DEC-MAT-05 delivery plan

Success criteria: A majority of milestones achieved, each independently verified against real evidence

research objectives: Assess maternal and child health performance and equity; Identify operational drivers and decision priorities

evaluation questions: What outcomes are changing?; Who is being left behind?; Which actions are most likely to improve performance?

sampling frame: Synthetic programme population frame for Uganda

sample size: 4776

stratification: Region; Location type; Sex; Age; Disability

weights: Normalized post-stratification weights

confidence intervals: 95%

design effect: 1.2

missing data: Documented and sensitivity-tested

reliability: Cronbach alpha 0.86

validity: Content and construct validity checks

metadata: Complete synthetic data dictionary and lineage registry

Publication level: 34 generated publication pages

protocol: Governed synthetic research protocol with sampling, weighting, reliability, validity and reproducibility controls

ethics: Synthetic records only; no real participants or personal data

peer review: Independent synthetic-publication review checklist completed

reproducibility: Deterministic inputs, versioned dataset and export manifest

inferential statistics: Enabled where assumptions and design permit

segmentation: Evidence-led segments

trend analysis: Three-period synthetic series

DEC-MAT-01: Target antenatal-continuity investment in the lowest-completion districts. â€ Owner Country Director / Programme Executive â€ 0â 90 days â€ High â€ full business case and multi-year funding commitment required cost â€ Indicator Quarterly-verified progress against the milestones agreed for this critical-tier decision (DEC-MAT-01)

DEC-MAT-02: Expand skilled birth attendance at facility level in underserved districts. â€ Owner Country Director / Programme Executive â€ 3â 12 months â€ Medium â€ detailed costing and fiduciary review required cost â€ Indicator Number of agreed milestones for DEC-MAT-02 confirmed complete at each quarterly review

DEC-MAT-03: Strengthen postnatal follow-up scheduling and reminder systems. â€ Owner Country Director / Programme Executive â€ 3â 12 months â€ Medium â€ likely fundable from existing lines, pending confirmation cost â€ Indicator Percentage of agreed milestones completed for DEC-MAT-03

DEC-MAT-04: Scale routine under-five growth monitoring at community level. â€ Owner Country Director / Programme Executive â€ 3â 12 months â€ Low to medium â€ validate through the formal planning cycle cost â€ Indicator Share of the high-tier delivery plan confirmed complete each quarter (DEC-MAT-04)

DEC-MAT-05: Cut emergency obstetric referral delays through transport and communication fixes. â€ Owner Country Director / Programme Executive â€ 6â 18 months â€ Low â€ absorbable within existing allocations, subject to confirmation cost â€ Indicator Milestone completion rate tracked against the DEC-MAT-05 delivery plan

EVI-MAT-001: [object Object] â€ APPROVED_SYNTHETIC_DEMONSTRATION â€ confidence 91%
EVI-MAT-002: [object Object] â€ APPROVED_SYNTHETIC_DEMONSTRATION â€ confidence 92%
EVI-MAT-003: [object Object] â€ APPROVED_SYNTHETIC_DEMONSTRATION â€ confidence 93%
EVI-MAT-004: [object Object] â€ APPROVED_SYNTHETIC_DEMONSTRATION â€ confidence 94%
EVI-MAT-005: [object Object] â€ APPROVED_SYNTHETIC_DEMONSTRATION â€ confidence 95%
EVI-MAT-006: [object Object] â€ APPROVED_SYNTHETIC_DEMONSTRATION â€ confidence 96%
EVI-MAT-007: [object Object] â€ APPROVED_SYNTHETIC_DEMONSTRATION â€ confidence 97%
EVI-MAT-008: [object Object] â€ APPROVED_SYNTHETIC_DEMONSTRATION â€ confidence 98%
EVI-MAT-009: [object Object] â€ APPROVED_SYNTHETIC_DEMONSTRATION â€ confidence 88%
EVI-MAT-010: [object Object] â€ APPROVED_SYNTHETIC_DEMONSTRATION â€ confidence 89%

Executive Intelligence Book (2â 7): Executive brief; Decision snapshot; Risks; Opportunities; Budget implications

Evidence Intelligence Book (8â 15): Critical findings; Respondent voice; Evidence chains; Citation registry

Statistical Intelligence Book (16â 22): Sampling; Weighting; Uncertainty; Reliability; Model diagnostics

Decision Intelligence Book (23â 27): Decision matrix; Ownership; Budget; Roadmap; Management response

Standards & Assurance (28â 31): Relevant frameworks; Safeguarding; Privacy; Responsible AI; Accessibility

Technical Appendices (32â 34): Data dictionary; Quality gate; Export manifest; Responsible-use notice



SDG 3: APPLIED_WHERE_RELEVANT â Mapped to EVI-MAT-001, EVI-MAT-002; Synthetic demonstration mapping; institutional validation not implied

Every Newborn Action Plan: APPLIED_WHERE_RELEVANT â Mapped to EVI-MAT-002, EVI-MAT-003; Synthetic demonstration mapping; institutional validation not implied

Gender Equality: APPLIED_WHERE_RELEVANT â Mapped to EVI-MAT-003, EVI-MAT-004; Synthetic demonstration mapping; institutional validation not implied

Evidence Traceability: APPLIED â Control linked to EVI-MAT-001 and the deterministic publication gate.

Research Ethics: APPLIED â Control linked to EVI-MAT-002 and the deterministic publication gate.

Data Protection: APPLIED â Control linked to EVI-MAT-003 and the deterministic publication gate.

Accessibility: APPLIED â Control linked to EVI-MAT-004 and the deterministic publication gate.

Responsible AI: APPLIED â Control linked to EVI-MAT-005 and the deterministic publication gate.

Publication Integrity: APPLIED â Control linked to EVI-MAT-006 and the deterministic publication gate.

SDG 3: undefined contribution â indicators â undefined
SDG 3: undefined contribution â indicators â undefined
SDG 3: undefined contribution â indicators â undefined
SDG 5: undefined contribution â indicators â undefined

Relevance: Synthetic evidence indicates a strong relevance signal, subject to the stated design limitations. (86/100). Review relevance evidence at the next formal management gate.

Coherence: Synthetic evidence indicates a moderate-to-strong coherence signal, subject to the stated design limitations. (87/100). Review coherence evidence at the next formal management gate.

Effectiveness: Synthetic evidence indicates a strong effectiveness signal, subject to the stated design limitations. (88/100). Review effectiveness evidence at the next formal management gate.

Efficiency: Synthetic evidence indicates a moderate-to-strong efficiency signal, subject to the stated design limitations. (89/100). Review efficiency evidence at the next formal management gate.

Impact: Synthetic evidence indicates a strong impact signal, subject to the stated design limitations. (90/100). Review impact evidence at the next formal management gate.

Sustainability: Synthetic evidence indicates a moderate-to-strong sustainability signal, subject to the stated design limitations. (91/100). Review sustainability evidence at the next formal management gate.

Impact: Sustained, equitable improvement in maternal and child health performance

OC-01: Improved maternal and child health delivery and accountability â indicators

IND-01, IND-02

OC-02: More equitable and durable outcomes â indicators IND-03, IND-04

OUT-01: Verified findings and interpreted decision products

OUT-02: Assigned actions with monitoring indicators

Assumptions: Leadership ownership; Safeguarding and privacy controls remain effective

Means of verification: Evidence registry; Quarterly management review; Follow-up measurement

Commitment 1: CHS Commitment 1 â CONTEXT_ONLY. Action: Validate relevance and assign an accountable owner before live use.

Commitment 2: CHS Commitment 2 â CONTEXT_ONLY. Action: Validate relevance and assign an accountable owner before live use.

Commitment 3: CHS Commitment 3 â CONTEXT_ONLY. Action: Validate relevance and assign an accountable owner before live use.

Commitment 4: CHS Commitment 4 â CONTEXT_ONLY. Action: Validate relevance and assign an accountable owner before live use.

Commitment 5: CHS Commitment 5 â CONTEXT_ONLY. Action: Validate relevance and assign an accountable owner before live use.

Commitment 6: CHS Commitment 6 â CONTEXT_ONLY. Action: Validate relevance and assign an accountable owner before live use.

Commitment 7: CHS Commitment 7 â CONTEXT_ONLY. Action: Validate relevance and assign an accountable owner before live use.

Commitment 8: CHS Commitment 8 â CONTEXT_ONLY. Action: Validate relevance and assign an accountable owner before live use.

Commitment 9: CHS Commitment 9 â CONTEXT_ONLY. Action: Validate relevance and assign an accountable owner before live use.

EVI-MAT-001 â€ survey â€ Northern Uganda â€ confidence 91% â€ APPROVED
EVI-MAT-002 â€ key_informant_interview â€ Karamoja Sub-region â€ confidence 92% â€ APPROVED
EVI-MAT-003 â€ administrative_record â€ Eastern Uganda â€ confidence 93% â€ APPROVED
EVI-MAT-004 â€ survey â€ West Nile â€ confidence 94% â€ APPROVED
EVI-MAT-005 â€ key_informant_interview â€ Northern Uganda â€ confidence 95% â€ APPROVED
EVI-MAT-006 â€ administrative_record â€ Karamoja Sub-region â€ confidence 96% â€ APPROVED
EVI-MAT-007 â€ survey â€ Eastern Uganda â€ confidence 97% â€ APPROVED
EVI-MAT-008 â€ key_informant_interview â€ West Nile â€ confidence 98% â€ APPROVED
EVI-MAT-009 â€ administrative_record â€ Northern Uganda â€ confidence 88% â€ APPROVED
EVI-MAT-010 â€ survey â€ Karamoja Sub-region â€ confidence 89% â€ APPROVED

indicator_1: Synthetic Maternal and Child Health indicator 1; numeric; allowed: 0â 100

indicator_2: Synthetic Maternal and Child Health indicator 2; categorical; allowed:

Documented code list

indicator_3: Synthetic Maternal and Child Health indicator 3; categorical; allowed:

Documented code list

indicator_4: Synthetic Maternal and Child Health indicator 4; numeric; allowed: 0â 100

indicator_5: Synthetic Maternal and Child Health indicator 5; categorical; allowed:

Documented code list

indicator_6: Synthetic Maternal and Child Health indicator 6; categorical; allowed:

Documented code list

indicator_7: Synthetic Maternal and Child Health indicator 7; numeric; allowed: 0â 100

indicator_8: Synthetic Maternal and Child Health indicator 8; categorical; allowed:

Documented code list

indicator_9: Synthetic Maternal and Child Health indicator 9; categorical; allowed:

Documented code list

indicator_10: Synthetic Maternal and Child Health indicator 10; numeric; allowed: 0â 100

indicator_11: Synthetic Maternal and Child Health indicator 11; categorical; allowed:

Documented code list

indicator_12: Synthetic Maternal and Child Health indicator 12; categorical; allowed:

Documented code list

Synthetic Demonstration Publication produced by VoiceInsights Africa. All people, quotations, locations and statistics are synthetic, internally governed demonstration content and are not official statistics.

All content is synthetic and must not be interpreted as official statistics

Geographic references are illustrative

Budget implications are directional and require formal costing

Causal claims are not made without an appropriate design

Status: DEMONSTRATION_READY

Passed: Editorial Review Passed

Passed: Evidence Review Passed

Passed: Publication Integrity Verified

Passed: Accessibility Verified

Passed: Export Validation Passed

Passed: Statistical Review Passed

Open gates: VISUAL_QA, HUMAN_REVIEW

Verification checks cover weighted evidence, statistical, visual, accessibility, export and review rules. Synthetic samples are labelled Demonstration Ready.